

SURGICAL PATHOLOGY REPORT

UID:CB491DC7-F20F-496F-A416-AA32105D32D7
TCGA-XF-ABHD-01A-PR

Redacted



Collected

Ordered by

FINAL DIAGNOSIS:

RADICAL CYSTECTOMY WITH CONTINENT URINARY DIVERSION:

RIGHT URETER (A):

FROZEN SECTION DIAGNOSIS:

- NEGATIVE

FINAL DIAGNOSIS:

- BENIGN URETERAL MUCOSA

- NO EVIDENCE OF MALIGNANCY

LEFT URETER (B):

FROZEN SECTION DIAGNOSIS:

- BENIGN URETERAL TISSUE

- NO EVIDENCE OF MALIGNANCY

URETHRAL MARGIN (C):

FROZEN SECTION DIAGNOSIS:

- NEGATIVE

FINAL DIAGNOSIS:

- BENIGN PROSTATIC GLANDULAR AND STROMAL TISSUE

- NO EVIDENCE OF MALIGNANCY

BLADDER AND PROSTATE (D):

BLADDER:

- INVASIVE UROTHELIAL CARCINOMA (4.0 x 3.5 x 2.5 CM), HIGH GRADE 4/4, INFILTRATING THROUGH THE MUSCULARIS PROPRIA AND APPROACHING TO THE PERI-VESSICLE FAT, BUT NOT AT THE INKED MARGIN
- LYMPHOVASCULAR SPACE INVASION IDENTIFIED
- URETHRAL, AND RIGHT AND LEFT URETERAL MARGINS, FREE OF MALIGNANCY
- EXTENSIVE ULCERATION, HEMORRHAGE AND NECROSIS PRESENT
- NO FLAT LESION (CARCINOMA IN SITU) IDENTIFIED
- NO MALIGNANCY IDENTIFIED IN THREE PERIVESICAL LYMPH NODES EXAMINED (0/3)

PROSTATE:

- MICROSCOPIC FOCUS OF PROSTATIC ADENOCARCINOMA, GLEASON' S SCORE 6 (3+3), INVOLVING LEFT MID PROSTATE
- NO PERINEURAL OR LYMPHOVASCULAR INVASION IDENTIFIED
- HIGH GRADE PROSTATIC INTRAEPITHELIAL NEOPLASIA (PIN II-III) IDENTIFIED
- BILATERAL SEMINAL VESICLES, FREE OF MALIGNANCY
- APICAL, BLADDER NECK AND INKED RADIAL MARGINS, FREE OF MALIGNANCY
- CHRONIC PROSTATITIS WITH GLANDULAR HYPERPLASIA AND ATROPHY

LEFT PARA-AORTIC LYMPH NODES (E): - NO MALIGNANCY IDENTIFIED IN 7 LYMPH NODES EXAMINED (0/7)

LEFT COMMON ILIAC LYMPH NODES (F):

- NO MALIGNANCY IDENTIFIED IN 6 LYMPH NODES EXAMINED (0/6)

RIGHT PARACAVAL LYMPH NODES (G):

- NO MALIGNANCY IDENTIFIED IN 8 LYMPH NODES EXAMINED (0/8)

RIGHT COMMON ILIAC LYMPH NODES (H):

- NO MALIGNANCY IDENTIFIED IN FIVE LYMPH NODES EXAMINED (0/5)

RIGHT LYMPH NODE OF CLOQUET (I):

- NO MALIGNANCY IDENTIFIED IN TWO LYMPH NODE EXAMINED (0/2)

ICD-O-3

Carcinoma, urothelial NOS
8120/3

Site: Bladder NOS
C67.9

9/5 11/23/14

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RIGHT EXTERNAL ILIAC LYMPH NODES (J):

- NO MALIGNANCY IDENTIFIED IN FIVE LYMPH NODES EXAMINED (0/5)

RIGHT OBTURATOR / HYPOGASTRIC LYMPH NODES (K):

- NO MALIGNANCY IDENTIFIED IN 18 LYMPH NODES EXAMINED (0/18)

LEFT LYMPH NODE OF CLOQUET (L):

- NO MALIGNANCY IDENTIFIED IN TWO LYMPH NODE EXAMINED (0/2)

LEFT EXTERNAL ILIAC LYMPH NODES (M):

- NO MALIGNANCY IDENTIFIED IN SEVEN LYMPH NODES EXAMINED (0/7)

LEFT OBTURATOR / HYPOGASTRIC LYMPH NODES (N):

- NO MALIGNANCY IDENTIFIED IN FOUR LYMPH NODES EXAMINED (0/4)

RIGHT PRE SCIATIC LYMPH NODES (O):

- NO MALIGNANCY IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

LEFT PRE SCIATIC LYMPH NODE (P):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1)

PRESACRAL LYMPH NODES (Q):

- BENIGN FIBROADIPOSE TISSUE
- NO LYMPHOID TISSUE OR MALIGNANCY IDENTIFIED

RIGHT PROXIMAL URETER (R):

- BENIGN URETERAL MUCOSA SHOWING NO HISTOPATHOLOGIC ABNORMALITY
- NO MALIGNANCY IDENTIFIED

LEFT PROXIMAL URETER (S):

- BENIGN URETERAL MUCOSA SHOWING NO HISTOPATHOLOGIC ABNORMALITY
- NO MALIGNANCY IDENTIFIED

PATHOLOGIC STAGE:

BLADDER: T3aN0MX

PROSTATE: T2aN0MX

TOTAL LYMPH NODE: 0/71

INTRAOPERATIVE DIAGNOSIS:

FROZEN SECTION DIAGNOSIS:

AFS: Right ureter
Negative

BFS: Left ureter
Negative

CFS: Ureteral margin
Negative

GROSS DESCRIPTION:

A: The specimen is received fresh from the O.R. for frozen section diagnosis and labeled "Right ureter". It consists of a segment of ureter with attached vas deferens measuring 0.3 cm in length and 0.4 cm in diameter. The specimen is frozen and entirely submitted in one cassette.

B: The specimen is received fresh from the O.R. for frozen section diagnosis and labeled "Left ureter". It consists of a segment of ureter with attached fat

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measuring 0.3 cm in length and 0.5 cm in diameter. The specimen is frozen and entirely submitted in one cassette.

C: The specimen is received fresh from the O.R. for frozen section diagnosis and labeled "Urethral margin". It consists of a tan-red tissue fragment measuring 2.2 x 0.6 x 0.3 cm. The specimen is frozen and entirely submitted in one cassette.

D: The specimen is received fresh from the O.R. and labeled "Bladder/prostate". It consists of a radical cystoprostatectomy specimen measuring overall 22.5 x 18 x 4 cm with smooth glistening peritoneum measuring 21 x 12.5 x less than 0.1 cm. The specimen is inked black from the margins and opened along the prostatic urethra to show the bladder 7 x 5 x 3 cm with uninvolved wall thickness of 0.8 cm. There is an ulcerative hemorrhagic mass involving the anterior bladder measuring 4 x 3.5 x 2.5 cm. Sections to this mass show it to involve the full thickness of the bladder wall with a depth of invasion of 2 cm. The remaining uninvolved bladder mucosa is tan-red and edematous. The ureters are easily found measuring 4 cm in length by 0.7 cm in circumference on the right and 3.5 cm in length by 0.7 cm in circumference on the left. The ureter mucosa is unremarkable except for a 0.3 cm polyp found on the left ureter. The tumor is above the ureteral vesicle junctions and extends to involve the right and left lateral bladder wall. The prostate itself measures 4 x 3.2 x 3 cm with prostatic urethra 3 cm in length by 3 cm in circumference and the verumontanum 0.8 x 0.3 x 0.3 cm. The prostatic urethral mucosa is hemorrhagic but smooth. Serial sections to the prostate show it to have a slightly nodular cut surface without grossly identifiable mass lesions. The right seminal vesicle measures 2.5 x 1.3 x 0.4 cm and the left seminal vesicle measures 2.3 x 1.3 x 0.6 cm. The perivesical fat is thinly sectioned to reveal no lymph nodes. Representative sections are submitted in 25 cassettes.

E: The specimen is received in formalin and labeled "Left para-aortic LN". It consists of two fragments of fibroadipose tissue measuring in aggregate 4 x 2.5 x 0.5 cm, containing multiple lymph nodes ranging in size from 0.5 to 1.5 cm in greatest dimension. The specimen is entirely submitted in three cassettes.

F: The specimen is received in formalin and labeled "Left common iliac LN". It consists of multiple fragments of fibroadipose tissue measuring in aggregate 4 x 2 x 1 cm, containing multiple lymph nodes ranging in size from 0.1 to 1 cm in greatest dimension. The specimen is entirely submitted in two cassettes.

G: The specimen is received in formalin and labeled "Right para-caval LN". It consists of a fragment of fibroadipose tissue measuring 5 x 3.5 x 0.9 cm, containing multiple lymph nodes ranging in size up to 1 cm in greatest dimension. The specimen is entirely submitted in two cassettes.

H: The specimen is received in formalin and labeled "Right common iliac LN". It consists of two fragments of fibroadipose tissue measuring in aggregate 3 x 2.2 x 0.6 cm, containing multiple lymph nodes ranging in size up to 1.5 cm in greatest dimension. The specimen is entirely submitted in two cassettes.

I: The specimen is received in formalin and labeled "Right LN of cloquet". It consists of a fragment of fibroadipose tissue measuring 2.5 x 1.5 x 0.4 cm, containing a lymph node measuring 1.5 cm in greatest dimension. The specimen is entirely submitted in one cassette.

J: The specimen is received in formalin and labeled "Right external iliac LN". It consists of multiple fragments of fibroadipose tissue measuring in aggregate 5 x 3.5 x 0.6 cm, containing multiple lymph nodes ranging in size up to 1.3 cm in greatest dimension. The specimen is entirely submitted in three cassettes.

K: The specimen is received in formalin and labeled "Right obturator/hypogastric LN". It consists of multiple fragments of fibroadipose tissue measuring in

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aggregate 8 x 5 x 1 cm, containing multiple lymph nodes ranging in size from 0.1 to 0.8 cm in greatest dimension. The specimen is entirely submitted in four cassettes.

L: The specimen is received in formalin and labeled "Left LN of cloquet". It consists of a lymph node with attached fat measuring 2 cm in greatest dimension. The specimen is entirely submitted in one cassette.

M: The specimen is received in formalin and labeled "Left external iliac LN". It consists of multiple fragments of fibroadipose tissue measuring in aggregate 3.5 x 2 x 0.5 cm, containing multiple lymph nodes ranging in size up to 1.1 cm in greatest dimension. The specimen is entirely submitted in two cassettes.

N: The specimen is received in formalin and labeled "Left obturator/hypogastric LN". It consists of multiple fragments of fibroadipose tissue measuring in aggregate 6 x 4 x 0.4 cm, containing multiple lymph nodes ranging in size up to 0.5 cm in greatest dimension. The specimen is entirely submitted in three cassettes.

O: The specimen is received in formalin and labeled "Right pre sciatic LN". It consists of multiple fragments of fibroadipose tissue and blood clot measuring in aggregate 2 x 1.5 x 0.3 cm. The specimen is entirely submitted in one cassette.

P: The specimen is received in formalin and labeled "Left pre sciatic LN". It consists of multiple fragments of fibroadipose tissue measuring in aggregate 1 x 0.8 x 0.3 cm. The specimen is entirely submitted in one cassette.

Q: The specimen is received in formalin and labeled "Pre sacral LN". It consists of a fragment of fibroadipose tissue measuring 5.6 x 2 x 0.2 cm. The specimen is entirely submitted in two cassettes.

R: The specimen is received in formalin and labeled "Proximal right ureter". It consists of a segment of ureter with attached fat measuring 1.6 cm in length and 0.4 cm in diameter. Two clips are located at one end, which is inked black. The specimen is serially sectioned and entirely submitted in one cassette.

S: The specimen is received in formalin and labeled "Proximal left ureter". It consists of a segment of ureter with attached fat measuring 1.5 cm in length and 0.5 cm in diameter. A clip is located at one end of the specimen and in the middle of the specimen. The site closest to the clip is inked black. The specimen is serially sectioned and entirely submitted in one cassette.

SECTIONS:

AFS: frozen section, right ureter
 BFS: frozen section, left ureter
 C FS: frozen section, ureteral margin
 D1: right ureter margin
 D2: left ureter margin
 D3: left ureter polyp
 D4: left ureteral vesicle junction
 D5: right ureteral vesicle junction
 D6-12: tumor, anterior bladder wall
 D13: trigone
 D14: dome
 D15: posterior wall
 D16: right distal prostate
 D17: left distal prostate
 D18: right mid prostate
 D19: left mid prostate
 D20: right mid prostate
 D21: left mid prostate

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D22: right seminal vesicle and vas deferens
D23: left seminal vesicle and vas deferens
D24-25: perivesical fat
E1-3: left para-aortic lymph node all embedded
F1-2: left common iliac lymph node all embedded
G1-2: right para-caval lymph node all embedded
H1-2: right common iliac lymph node all embedded
I: right lymph node of cloquet all embedded
J1-3: right external iliac lymph node all embedded
K1-4: right obturator/hypogastric lymph node all embedded
L: left lymph node of cloquet all embedded
M1-2: left external iliac lymph node all embedded
N1-3: left obturator/hypogastric lymph node all embedded
O: right pre sciatic lymph node all embedded
P: left pre sciatic lymph node all embedded
Q1-2: pre sacral lymph node all embedded
R: proximal right ureter all embedded
S: proximal left ureter all embedded

MICROSCOPIC DESCRIPTION:

A-C: See final microscopic-diagnosis.

D: Sections of the bladder show sheets of high-grade malignant cells infiltrating through the muscularis propria and approaching to the peri-vesicle fat, but not at the inked margin. The nuclei of these cells are hyperchromatic with high N/C ratio and indistinct to prominent nucleoli (D6-12). A high ratio of apoptotic activity is noted as well as increased mitotic activity (2 mitotic figures per 10/phf). Lymphovascular space invasion is identified. Margins are free of malignancy.

Sections of the prostate show a microscopic focus of malignant glands infiltrating into the stroma (D19). These neoplastic cells show high N/C ratio with nuclei showing increased hyperchromatism and distinct nucleoli. In addition, multifocal high-grade prostatic intraepithelial neoplasia (PIN II-III) is identified (D19-21). The remaining prostatic gland shows extensive glandular hyperplasia, chronic inflammation and atrophy.

E-S: See final microscopic-diagnosis.

Signed by:

ELECTRONIC SIGNATURE

PRE-OPERATIVE DIAGNOSIS:

Bladder cancer

ORDERING PHYSICIAN:

Ordering Physician:

Y/R/r/s	hw 10/21/13	Yes	No
Diagnosis Discrepancy			
Primary Tumor Site Discrepancy	Prostate		
I/PAA Discrepancy			
Prior Malignancy History			
Qual/Synchronous Primary Noted			
Case is (circle):	QUALIFIED	DISQUALIFIED	
Reviewer Initials	JHC	Date Reviewed:	10/24/2013